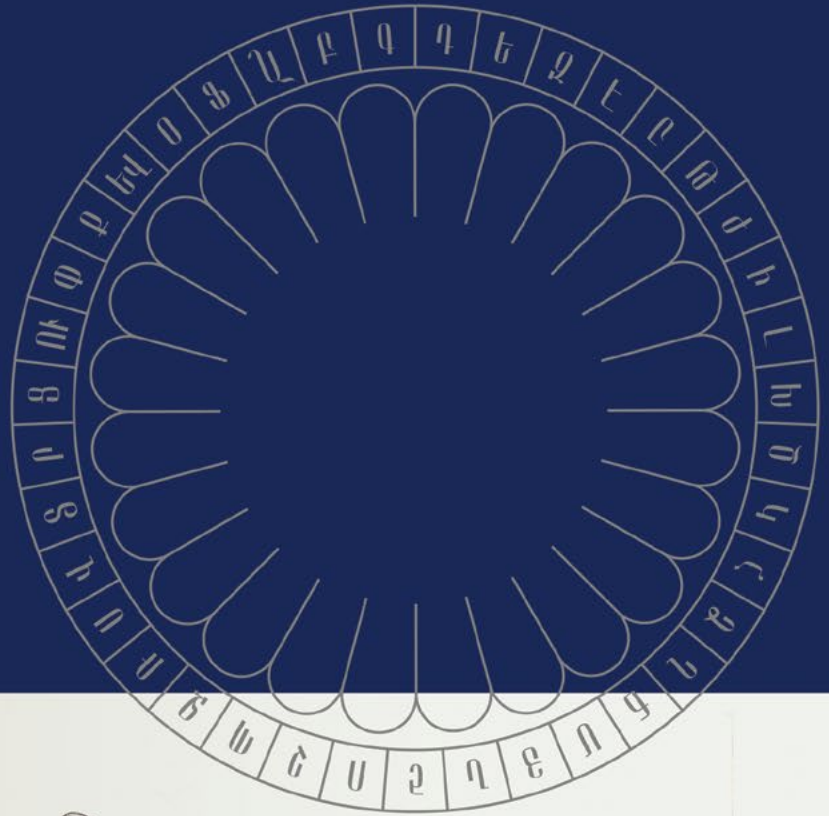


WHO Country Cooperation Strategy

Armenia
2026–2030



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Նախարարություն
Ministry of Health
of the Republic of Armenia



World Health
Organization

European Region



Document number: WHO/EURO:2026-13205-52979-82603 (PDF); WHO/EURO:2026-13205-52979-82602 (print)

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Armenia
2026–2030



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Նախարարություն
Ministry of Health
of the Republic of Armenia



**World Health
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European Region



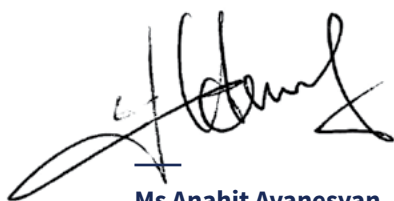


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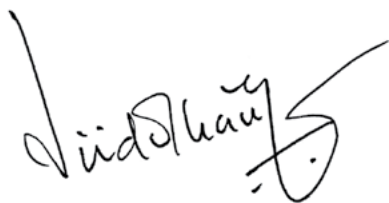
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Foreword



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Minister of Health
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WHO Representative and Head
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WHO Regional Director for Europe

The Ministry of Health of the Republic of Armenia and the World Health Organization are proud to present the WHO–Armenia Country Cooperation Strategy (CCS) for 2026–2030. This strategy charts a bold and collaborative path to advance the health and well-being of all people in Armenia, resulting in inclusive and sustainable human capital and economic development.

Developed in close alignment with Armenia’s national health and development agenda, the CCS charts a strategic framework for partnership between Armenia and WHO. It underscores health as a cornerstone of sustainable development and peace, guiding joint efforts to advance health equity, strengthen health systems, bolster health security, secure demographic resilience, improve population well-being and tackle the health impacts of climate change.

While navigating the aftermath of the coronavirus disease pandemic, growing health and social impacts from climate change, and complex socioeconomic shifts, this strategy is a forward-looking commitment to ensure every citizen in Armenia – regardless of age, gender or geographical location – lives a healthy and productive life. This commitment is harmonized with the WHO Fourteenth Global Programme of Work, 2025–2028 and the Second European Programme of Work, 2026–2030. Central to this CCS is the ambition to build a sustainable, people-centred health system that delivers integrated, accessible and affordable care for all people whenever and wherever they need it so that no one is left behind.

By working closely with the Government, WHO will support the implementation of Armenia’s national health goals and address evolving health and well-being needs in Armenia, thereby advancing progress towards achieving the Sustainable Development Goals.

We extend our gratitude to all who contributed to this strategy and warmly invite every stakeholder – from health professionals and community leaders to national and international partners and citizens – to engage with this vision. Together, we can build a healthier, more equitable future for Armenia.



Abbreviations

COVID-19	coronavirus disease
CCS	Country Cooperation Strategy
EPW2	Second European Programme of Work 2026–2030
GPW 14	WHO's Fourteenth General Programme of Work, 2025–2028
NCDs	noncommunicable diseases
SDGs	Sustainable Development Goals
SFAs	strategic focus areas
TB	tuberculosis
UN	United Nations
UNSDCF	United Nations Sustainable Development Cooperation Framework





Introduction

The development of this Country Cooperation Strategy (CCS) between WHO and Armenia for 2026–2030 marks a transformative repositioning of WHO’s partnership in Armenia. With a strategic vision aligned with national, regional and global health priorities, the CCS undertakes a shift from the 2-year operational planning under the Biennial Collaborative Agreement, to a 5-year horizon, driving long-term public health impact. The CCS underpins sustainable impact, whole-of-WHO engagement and alignment with Armenia’s national priorities, including the Government of Armenia and the United Nations Sustainable Development Cooperation Framework (UNSDCF) 2026–2030 (1), which provides an opportune platform to craft a strategic, impactful and targeted investment by WHO in Armenia and to foster collaborative work to respond to the health and well-being needs of the population. This CCS acts as a framework to translate WHO’s Fourteenth General Programme of Work, 2025–2028 (GPW 14) (2) and the Second European Programme of Work 2026–2030 (EPW2) (3) into the country context.

To advance the United Nations (UN) 2030 Agenda for Sustainable Development in Armenia and mainstream the health of all people as a prerequisite for peace and security, the CCS is developed through broad multisectoral consultation. The CCS leverages WHO’s technical expertise across all three levels of the Organization (the Country Office in Armenia, the Regional Office for Europe and WHO headquarters) in addressing the social, economic and commercial determinants of health, aiming to promote health, deliver quality services and protect the well-being of all in Armenia. The CCS also reinforces WHO’s commitments towards gender equality and the prevention of and response to sexual misconduct. By embedding survivor-centred safeguards and gender-responsive approaches, the CCS will support Armenia in creating a resilient and holistic health system that protects rights, strengthens accountability and responds equitably to the needs of its citizens and especially vulnerable groups¹ – including refugees from Karabakh.

The CCS builds upon the relevance and coherence of past interventions and WHO’s work in Armenia in line with the findings from an independent evaluation carried out in 2024: while recognizing the solid foundation of WHO’s work in Armenia, the evaluation highlighted key areas to further amplify WHO’s impact – expanding strategic partnerships and engagement with UN and non-UN entities, reinforcing resource mobilization to ensure the sustainability of WHO-supported initiatives and increasing the visibility of WHO’s mandate in Armenia.

Reinforcing the key role of health as a driver of peace and sustainable development, the CCS will deliver tangible benefits in the lives of the Armenia population, irrespective of their geographic location or economic status. The CCS will also serve as a blueprint to ensure resilient health security, support and well-being, contributing towards stronger human capital through a healthy life and improved productivity. As the discourse around health and development evolves in Armenia – encompassing demographic resilience,

1 People or groups in vulnerable and marginalized situations can include children and adolescents; women and girls; people with disabilities; migrants, refugees and asylum-seekers; lesbian, gay, bisexual, transgender and intersex people; and older people.

health security, and climate and health integration, among other aspects – the CCS’s five-year horizon is designed to be “fit-for-purpose” and will be adaptable and responsive to the evolving health-care needs in shaping the future of health and well-being in Armenia. It also serves as a catalytic platform for resource mobilization and multisectoral action to address varied determinants of health and measurable impact towards a healthier Armenia for today and for tomorrow.

The Ministry of Health will play a leading role in ensuring “equitable, quality and resilient health” in Armenia in collaboration with other relevant ministries. Recognizing the need to address social, economic, physical, mental and behavioural determinants and risk-factors of health, the CCS will be an enabling platform to foster multisectoral engagement in ensuring a demonstrable impact on population health in Armenia.



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Situation analysis

Country context

Armenia is a landlocked country located in the south Caucasus with approximately 3 million residents. It is a parliamentary republic, with the Prime Minister as the head of government and the President as the head of State. The World Bank has classified Armenia as an upper-middle-income country (4) and with a Human Development Index of 0.811, Armenia ranks 69th globally in human development, placing the country in the high human development category (5). The UN estimates a 15% decrease in the total population of Armenia to under 2.5 million by 2050 (6). More than one-third of people reside in the capital, Yerevan (7). The population is ethnically homogenous, and more than 98% of people living in the country are Armenians (4). The Armenian diaspora is significant, with an estimated 7 million living in more than 100 countries around the world (8). Armenia's 2024–2040 Demographic Strategy focuses on population growth, family-friendly policies, attracting skilled labour, and investing in human capital to drive economic growth and sustainability (9).

At the end of 2023, Armenia's poverty rate was 23.7%, down 1.1 percentage points from 2022, while extreme poverty was at 1.1%; a 0.1-point decrease. Poverty affected 31.7% of children, with disparities based on gender, age and location. Extreme poverty impacted 1.9% of girls and 1.7% of boys (10).

Armenia is a mountainous country with a continental climate. It is prone to natural disasters, including earthquakes and floods, with both the 1988 Spitak earthquake and the 2024 Lori flood fresh in collective memory. Armenia faces significant environmental challenges, including deforestation, air pollution and water stress. A significant proportion of the population is vulnerable to flooding and drought, and these risks are likely to increase in the future with climate change. Without adaptation, these effects will deepen economic and social inequality, hitting poorer rural communities hardest (11).

Armenia has advanced major economic and political reforms while addressing the challenges of the coronavirus disease (COVID-19) pandemic, conflict, the influx of more than 120 000 ethnic Armenian refugees from Karabakh, natural disasters and external pressures. Moving forward, the Government's intention to conclude a peace agreement with Azerbaijan represents a key opportunity to secure stability and unlock new avenues for development and growth. Anchored within the Government's "Crossroads of Peace" initiative that positions Armenia to strengthen its economy, expand regional cooperation and marshal greater investments into health and human development (12).

Health situation

Since Armenia became an independent state in 1991, its health system has transitioned from a centralized Soviet-era model to a decentralized structure. However, persistent financial, structural and workforce-related challenges continue to affect the accessibility and quality of health-care services across the country. The burden of out-of-pocket expenditures remains significantly high, while disparities in health-care access between urban and rural areas pose additional obstacles to achieving universal health coverage. Yerevan suffers the poorest air quality in the south Caucasus (13), with air pollution causing 2192 premature deaths in 2016 – mainly from ischemic heart disease – among the highest rates in the WHO European Region (14).

The national health system, workforce and financing

The Armenian health-care system operates under a mixed model, with both public and private providers delivering health services. On an average, Armenians are within 14 minutes of their nearest primary health care facility. Despite the geographical availability of primary health care facilities, utilization remains low in certain groups of the population. The main factors affecting utilization according to a recent World Bank study were the unavailability of services (reported by 15.1% of respondents), concerns about the quality of care (10.8%) and the cost of services (8.2%) (15).

Armenia's health-care workforce is characterized by an imbalance in the distribution of health professionals, with 74% of doctors based in Yerevan, while rural and regional health-care facilities face chronic shortages of medical personnel. An aging workforce further exacerbates the situation, with 22.3% of doctors and 17.7% of nurses aged 55 years or older in 2024 (16). This demographic trend calls for strategic investments in training and retention programmes.

Overall government health expenditure remains critically low, accounting for only 2.2% of gross domestic product in 2021 – one of the lowest rates in the WHO European Region. This has resulted in a significant reliance on out-of-pocket payments, which accounted for approximately 78.7% of total health spending in 2021 (16) placing financial strain on households and limiting equitable access to essential health services, particularly for vulnerable populations. The Government has prioritized expanding universal health coverage and strengthening primary health care as key strategies to improve financial protection and service delivery, and has steadily expanded the basic benefits package to include more primary health care services and cover more people. However, major barriers remain, including inadequate public funding, fragmented service delivery and the absence of a comprehensive health insurance system. While efforts have been made to improve financial risk protection, particularly for vulnerable groups, the overall health financing landscape requires further reforms to ensure sustainability and equitable coverage. The ultimate goal is to implement a universal health coverage system that would facilitate access to quality health-care services to the entire population.

Health indicators and challenges

Despite notable improvements in health outcomes, Armenia continues to face challenges in addressing disparities and improving overall population health.

- **Life expectancy:** the average life expectancy in Armenia improved by 4.8 years from 72.9 in 2000 to 77.7 in 2023 (17,18).
- **Infant and maternal mortality:** infant mortality in Armenia dropped considerably between 2000 and 2022 from 15.6 to 6.7 per 1000 live births – slightly above the WHO European Region average. Maternal mortality also declined from 50.2 per 100 000 live births in 2000 to 27.2 in 2020 – which is still more than twice the WHO European Region average (18). Routine childhood vaccinations rates remain high; with 96% of children receiving the first and second doses of measles-containing vaccine (19) – higher than the WHO European Region average (20).
- **Noncommunicable diseases (NCDs):** the four most prevalent NCDs account for 77.0% of total deaths in Armenia. The leading causes of NCD death are cardiovascular diseases (53.5%) and cancer (20.8%). Tobacco use and dietary risk factors continue to substantially contribute to Armenia's NCD burden (21).
- **Mental health:** Armenia has a high prevalence of depression, particularly among populations affected by conflict and displacement. The demand for community-based and intersectoral mental health services has increased, yet gaps remain in service availability, accessibility and in the integration of mental health into primary health care (22,23). Strengthening rights-based and community-based mental health and psychosocial services that address the needs of vulnerable groups is critical.
- **Communicable diseases:** communicable diseases account for 11.8% of total deaths in Armenia. The incidence of new tuberculosis (TB) cases has steadily declined to 27 per 100 000 population in 2021 while the proportion of multidrug-resistant TB cases among newly detected cases has increased from 15% in 2015 to 18% in 2022.² Regarding HIV, the percentage of people living with HIV who knew their status was estimated at 76% in 2024. Of these people, 80% were on antiretroviral treatment, and 86% of those on antiretroviral treatment had achieved viral suppression (24). Armenia remains a high burden viral hepatitis country, with estimates indicating a hepatitis B virus incidence of 0.2 and a hepatitis C virus incidence of 0.3 per 100 000 inhabitants in 2021.³ Regarding sexually transmitted infections, Armenia has seen a marked rise in reported syphilis cases in recent years, driven by – among others – improved case detection and the availability of free treatment since 2020.⁴

Impact of the COVID-19 pandemic and the refugee crisis

The COVID-19 pandemic placed unprecedented strain on Armenia's health system, exposing gaps in health infrastructure, workforce capacity and emergency preparedness. WHO played a central role in supporting the country's pandemic response by providing medical supplies, financial assistance and technical guidance. However, systemic challenges – including limited intensive care capacity and delays in vaccine rollout – underscored the need for further investments in health security and preparedness.

In 2023, Armenia faced an additional public health challenge following the influx of ethnic Armenian refugees from Karabakh. WHO was actively involved in coordinating emergency health responses, particularly in mental health and psychosocial support, immunization campaigns and strengthening primary health care services for displaced populations. While government policies ensured universal health access for refugees, service delivery constraints – especially in rural areas – posed significant challenges in meeting the increased demand for health care.

2 WHO Regional Office for Europe. Comprehensive Review of The National Tuberculosis Control Programme in Armenia 22 – 31 May 2019 Mission Report. Unpublished report, 2019.

3 Data provided by the National Center for Disease Control and Prevention, Armenia, unpublished data, 2022.

4 Data provided by the National Center for Infectious Diseases, Armenia, unpublished data, 2021.

National health and development agenda

The health policy document in Armenia – the Health System Development Strategy (2023–2026) (25) – prioritizes six key areas: maternal and child health, NCDs, communicable diseases, mental health, safe environments and geriatric care. The targets to be achieved by 2026 include increasing life expectancy to 76 years, reducing NCD mortality by 1.5–2% annually, cutting maternal mortality to 16 per 100 000 live births, lowering under-5 mortality to 8.2 per 1000 live births, reducing the incidence of controllable infectious diseases to zero and achieving the HIV/AIDS 90-90-90 targets.⁵ Strategic approaches to improve maternal and child health focus on prenatal care, safe deliveries, neonatal services and mental health support for adolescents. NCD prevention emphasizes healthy lifestyles, early detection and expanding rehabilitation. Communicable disease control prioritizes vaccination, hepatitis C virus/HIV/TB elimination and epidemic preparedness. Mental health reforms emphasize the development of rights- and community-based services, while advancing the integration of mental health into primary health care. Safe environments targets injury reduction, traffic safety and climate adaptation. Finally, geriatric care integrates medical and social services, dementia management and nursing home regulation (25).

The Health System Development Strategy (2023–2026) aligns with GPW 14 by responding to climate change via safe environment initiatives; addressing health determinants through tobacco control and maternal/geriatric care; advancing primary health care for universal health coverage via insurance and e-health systems; improving financial protection with cost-reduction targets; and preparing for emergencies through communicable disease control (2). The Health System Development Strategy reflects the EPW2 megatrends through confronting NCDs and mental health (e.g. smoking reduction, deinstitutionalization); ageing is living (e.g. geriatric health integration); climate change (e.g. safe environments); and health systems of the future (e.g. e-health, primary health care expansion), while also recognizing the importance of equity and gender-responsive approaches (3).

The 2025–2028 Strategy for the Implementation of Gender Policy in the Republic of Armenia (26) addresses a number of issues related to the health-care system, defining specific measures aimed at solving them. Priority 4 of the Strategy ensures equal opportunities for women and men in the health-care sector. These measures are envisaged to increase women's access to basic health care and reproductive health-care services, especially for vulnerable groups, women from rural areas, women with disabilities and women belonging to ethnic minority groups. This includes increasing allocations from the state budget and including these groups in the compulsory health insurance system.

Programme of the Government of the Republic of Armenia (2021–2026)

Within the Programme of the Government of the Republic of Armenia (2021–2026) (27), health care is set out as one of the key objectives. Strategic efforts will focus on universal access, prevention activities and the digital modernization of the health system to ensure equitable, high-quality care. A comprehensive health insurance system aims to cover outpatient services and specialized diagnostics, aiming to cut private health spending by 40% while introducing quality control mechanisms. Prevention efforts focus on the early detection of infectious and noncommunicable diseases, expanded screening programmes (e.g. for breast cancer), and free treatment for chronic conditions such as diabetes and hepatitis C. Rare diseases in children will receive targeted drug coverage. Healthy lifestyle campaigns aim to reduce NCD

5 The Joint United Nations Programme on HIV and AIDS 90-90-90 targets were a set of goals to help end the AIDS epidemic, aiming for: 90% of people living with HIV to know their status, 90% of those diagnosed to receive antiretroviral therapy and 90% of those on antiretroviral therapy to achieve viral suppression.

mortality by 1.5% annually, addressing diet and youth health. Maternal and child health initiatives include free prenatal care, adolescent reproductive screenings and vaccinations, targeting a 15% drop in infant mortality and 18% reduction in maternal deaths. A fully integrated e-health system will streamline appointments, e-prescriptions and sick leave management, with a goal of five times more providers joining the platform. Emergency preparedness includes laboratory strengthening, real-time disease monitoring and pandemic-proofing measures. Medical education reforms and salary increases tied to performance will retain skilled professionals, alongside licencing systems for practitioners. The Programme emphasizes cross-sector collaboration and resilient infrastructure to address disparities, particularly in rural areas (27).

UNSDCF for Armenia (2026–2030)

The UNSDCF for Armenia (2026–2030) aligns with national and global agendas, focusing on four key pillars. The “Human Capital & Wellbeing” pillar prioritizes equitable access to quality education, health care and social protection, emphasizing digitalization, universal health coverage and demographic resilience through family-friendly policies. The “Inclusive Economic Development” pillar fosters green growth, decent work and private sector collaboration, targeting vulnerable groups via livelihood programmes, food security, women’s economic empowerment and infrastructure upgrades. The “Green Transition for Resilience” pillar advances climate change adaptation and mitigation through low-carbon policies, biodiversity conservation, disaster risk management and public awareness campaigns. The “Governance for a Sustainable Future” pillar aims to strengthen institutions by enhancing transparency, anti-corruption measures and human rights protections, including justice reform, migration management and inclusive civic participation, particularly for women, youth and marginalized groups. Cross-cutting themes like gender equality, shock responsiveness and social cohesion underpin all outcomes, aiming to reduce inequalities, boost resilience and accelerate progress toward the Sustainable Development Goals (SDGs) by 2030. Through partnerships, the UN will support Armenia in balancing economic transformation with environmental sustainability and equitable governance, ensuring no one is left behind (1).

Partnerships and collaborations

Armenia has fostered a strong partnership environment in health, with the Government leading key reforms and actively advancing its global and regional commitments. Armenia integrates health priorities into national strategies and action plans, ensuring alignment with global development agendas. The Ministry of Health – in close cooperation with other relevant ministries and together with the WHO Country Office in Armenia – engages and cooperates with a broad platform of national and international partners such as UN agencies, international and multilateral financial institutions, as well as with professional associations, civil society, academia, communities, youth and other interested parties. This inclusive approach not only drives the implementation of reforms that are people and community focused but also strengthens national ownership and resilience. Looking ahead, the partnership framework will aim to be further reinforced through sustainable financing, participatory mechanisms, innovation and population resilience, enabling Armenia to accelerate progress towards human capital development, well-being and health system resilience as a holistic one. At the same time, it will be positioned to capitalize on new opportunities under the Government’s flagship “Crossroad of Peace” initiative, reinforcing the role of health in peace and development in the Region.



Strategic focus areas (SFAs) and key deliverables

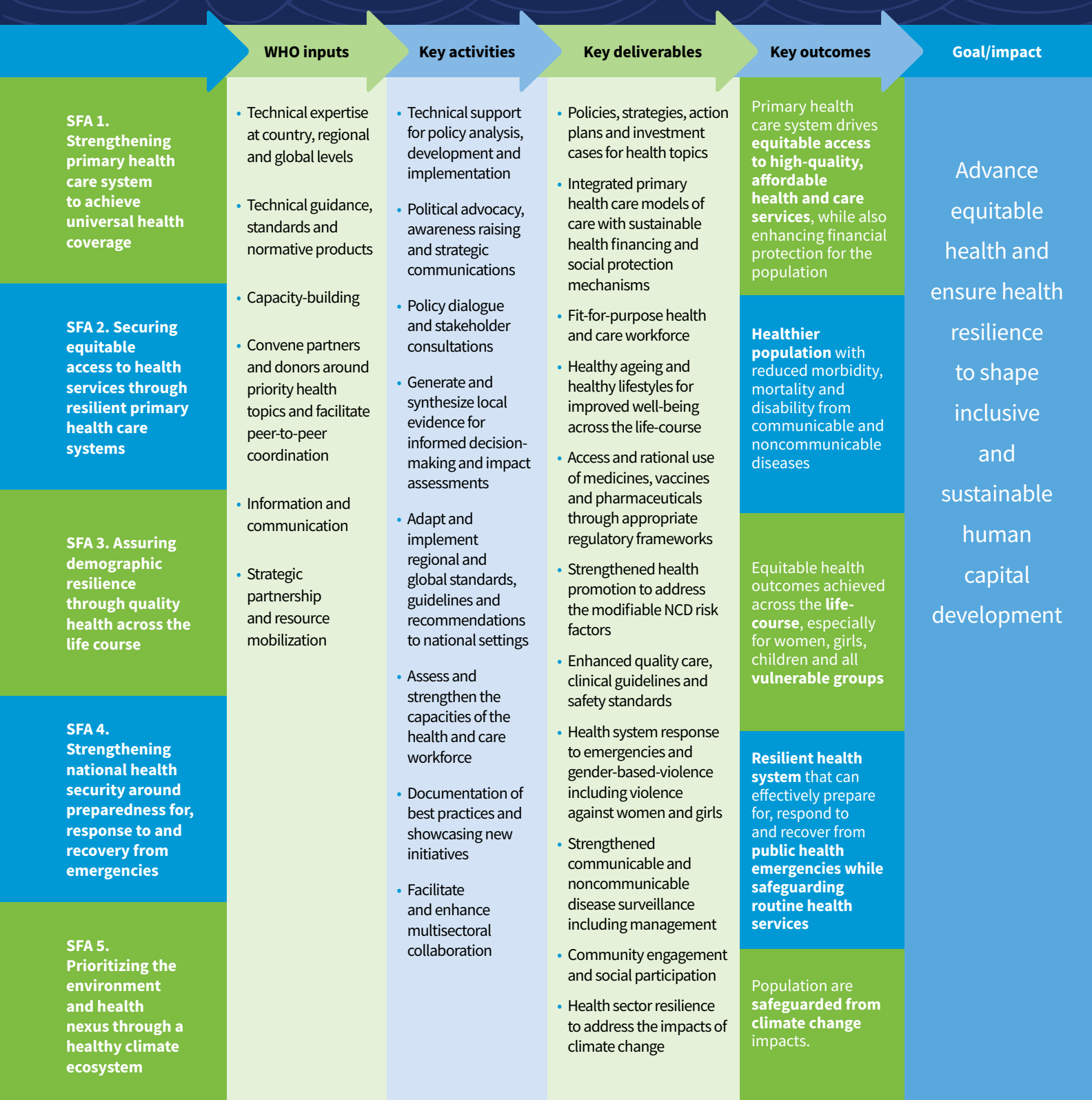
Recognizing health as a central pillar of Armenia's development agenda, this CCS will contribute to a productive society and thriving economy. The SFAs that form the structure of the CCS reflect GPW 14 and EPW2 outcomes that the Ministry of Health considers to be of highest priority for Armenia (Annex). These impact areas – outlined in turn below – will advance human capital development in the country, thereby accelerating the achievement of SDG3 and contributing to other SDGs. Aligned with global and regional strategies and indicators, this 2026–2030 CCS monitoring and evaluation framework will serve as an accountability compass to objectively measure progress. This will include a collaborative mechanism to identify areas needing improvement.

A theory of change presented in Fig. 1 provides a comprehensive, logical roadmap explaining how WHO will assist Armenia in meeting its key health goals.



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Fig. 1. Theory of change



Enabling principles: Ensuring “leaving no one behind” | Addressing gender and equality | Enhancing digital transformation

Assumptions

Political commitment; “health” remains central to the development agenda; enhanced and sustained intersectoral collaboration; adequate resources and fundraising opportunities; strategic partnerships and community engagement; gender, disability and rural–urban equity lenses in health policies; a gender and equity-sensitive approach and a focus on vulnerable population groups; and continuous monitoring, evaluation and learning.

Risks

A lack of resources (financial and human); emergencies (new and re-emerging) stressing the health system; a lack of engagement from health providers and stakeholders, including the population; slow adoption of new initiatives; and fragmented intersectoral coordination.

SFA 1. Strengthening the primary health care system to achieve universal health coverage

The ecosystem of high out-of-pocket spending and low public health funding in Armenia creates financial hardship and limits access to health-care services, particularly for vulnerable groups. By advancing universal health coverage through a strengthened primary health care system (e.g. access to quality preventive, curative, rehabilitative and palliative health and mental health services) for everyone regardless of their income status, gender or geographical location, Armenia's population will be able to direct their household budget (otherwise used for out-of-pocket expenses for health care) towards education and entrepreneurship, thereby fostering economically productive future generations and resilience.

Key deliverables

WHO will support the country to:

1. accelerate the implementation of a comprehensive primary health care reform strategy with a focus on standards and governance around quality-of-care and service reorganization including integration with essential public health functions;
 2. establish and implement a national health insurance system, with a focus on appropriate benefit packages, sustainable health financing, strengthened regulation of medicine pricing and reimbursement;
 3. design and implement equitable health financing strategies to reduce out-of-pocket expenditures, expand population coverage and target financial risk protection for vulnerable groups;
 4. advance context-specific health workforce planning, including capacity-building on workforce forecasting, the development of appropriate licencing and regulatory frameworks, and the implementation of training and retention strategies;
 5. design and implement context-specific integrated health and care services, including sensory care across the life course with a focus on people with disabilities, older people and people with chronic conditions, and covering prevention, early intervention, long-term care, rehabilitation and palliative care;
 6. strengthen access to quality-assured essential medicines and health technologies through regulatory strengthening, supply chain optimization, rational use of medicines and transparent pricing and reimbursement systems; and
 7. design and implement a digital health strategy with appropriate digital tools and health information systems, including predictive modelling, the analysis and use of data for decision-making, and to track progress and trends on health outcomes.
-



SFA 2. Securing equitable access to health services through resilient primary health care systems

Robust health-care systems that reach everyone equitably lay the foundation for sustainable development. Despite progress in life expectancy, Armenia faces a few critical health challenges. NCDs drive 83% of deaths, with tobacco use, alcohol consumption, physical inactivity, unhealthy diets and air pollution being key risk factors. Road traffic injuries are the leading cause of premature mortality among children and young adults in Armenia, with rates approaching twice the WHO European Region average (28). Mental health and psychosocial services remain inadequate, and communicable diseases like TB, HIV, viral hepatitis and vaccine-preventable diseases require timely detection and adequate treatment and care systems. Antimicrobial resistance and the growing burden of NCDs often hit vulnerable populations the hardest in Armenia. Through a resilient primary health care system and development of community based and intersectoral social-health-care services capable of tackling medical and social determinants of health, Armenia is committed to ensuring that the population has access to quality services.

Key deliverables

WHO will support the country to:

1. strengthen immunization across the life course with a focus on addressing inequities in immunization coverage, generating demand, introducing new vaccines and ensuring preparedness for and response to vaccine-preventable diseases;
 2. control and eliminate priority communicable diseases through integrated management with a focus on TB control and elimination; HIV prevention, testing, treatment and elimination; hepatitis prevention and treatment; and management of sexually transmitted infections;
 3. control antimicrobial resistance through improved antimicrobial stewardship, clinical governance, education and awareness raising, guidelines and diagnostics;
 4. accelerate intersectoral implementation of the best (29) and quick (30) buys of NCDs by addressing modifiable risk factors to address social and commercial influences and determinants of NCDs;
 5. strengthen population-based NCD surveillance systems and reduce mortality from NCDs through access to timely and effective health-care interventions and NCD management; and
 6. strengthen equitable access to mental health services using primary health care and community-based comprehensive care as a platform to address risk factors and the social determinants of mental ill-health.
-



SFA 3. Assuring demographic resilience through quality health across the life course

Demographic resilience allows a healthy and productive population across the life course to contribute and benefit from society's long-term prosperity. By building a demographically resilient society, Armenia will further strengthen the provision of quality health and care services that empower citizens at every life stage, from childhood through to older age. This will include sustained investment in quality maternal, child and adolescent health care, implementing healthy aging initiatives that combat ageism, and addressing sexual and reproductive health to empower women and enhance response to gender-based violence including violence against women and girls. These approaches will support longer, healthier, socially cohesive lives while fostering economic growth and stability for future generations.

Key deliverables

WHO will support the country to:

1. develop and implement national child and adolescent health policies ensuring equitable access to high-quality care with targeted actions and evidence-informed strategic investment to circumvent harmful commercial influences;
 2. strengthen quality maternal and newborn health care in primary health care and hospital settings with a focus on addressing the high caesarean section rates;
 3. develop and implement national healthy ageing policies to support longer, healthier lives in supportive environments and tackle ageism by prioritizing person-centred, gender-sensitive and dignity-based approaches to prevention and care transformation; and
 4. strengthen the health system response to gender-based violence and violence against women and girls, including through the integration of survivor-centred clinical protocols, referral pathways and training for health workers.
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SFA 4. Strengthening national health security around preparedness for, response to and recovery from emergencies

Recent emergencies like the COVID-19 pandemic, floods and the Karabakh refugee crisis have exposed vulnerabilities in Armenia's health security systems, revealing gaps in surveillance, infrastructure and cross-sector coordination. Investing in and institutionalizing preparedness by strengthening emergency systems and applying a whole-of-government, all-hazards approach (including the One Health domain) will enable Armenia to mount a timely and coordinated response to future health emergencies. This will save lives, safeguard economies from catastrophic disruption and maintain stability in communities during crises. Robust emergency response systems with gender-sensitive and safeguarding measures help maintain essential health services, keep businesses open, preserve infrastructure and maintain public confidence in institutions while protecting vulnerable populations from physical and mental health consequences.

Key deliverables

WHO will support the country to:

1. implement a multiple-hazard, gender-sensitive national health security action plan and Pandemic Agreement (when it comes into force) through the assessment of International Health Regulation core capacities and functionality of health facilities;
 2. enhance collaborative event- and indicator-based surveillance by strengthening early warning systems to rapidly identify in-country and cross-border health threats;
 3. conduct an impact analysis and public health risk assessments of acute emergencies, including multisectoral coordination for a survivor-centred response and referrals to address physical, social and mental health consequences;
 4. strengthen laboratory networks for preparedness, rapid detection and response to (re-)emerging cross-border threats and high-threat pathogens by enhancing pathogen sequencing and data-sharing mechanisms;
 5. enable social participation and community partnerships as part of emergency preparedness and response, strengthening risk communication, behavioural insights and the response to mis- and disinformation; and
 6. strengthen clinical governance on infection prevention and control in health-care settings, including improved policy around surveillance for safe and scalable care.
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SFA 5. Prioritizing the environment and health nexus through a healthy climate ecosystem

A strong environment–health nexus allows communities and economies to thrive for generations. Armenia’s environmental challenges – from air pollution to intensifying heatwaves and floods – disproportionately impact rural and marginalized communities, deepening health inequities. These environmental threats compound existing vulnerabilities in the communities and create new risks such as climate-aggravated mental health burdens and gendered impacts. Climate-smart, people-centred health systems reduce the burden on hospitals while creating co-benefits: cleaner air means more productive workforces, sustainable water management prevents both disease and agricultural losses, and heat-resistant infrastructure safeguards populations and strengthens societal resilience. Multisectoral collaboration and adaptive health systems will mitigate the cascading health consequences of environmental degradation in Armenia while protecting its vulnerable citizens.

Key deliverables

WHO will support the country to:

1. generate awareness and facilitate political advocacy for robust national actions to address the health impacts of climate change, including through the implementation of climate-health strategies;
 2. strengthen health sector resilience in addressing the environmental determinants of health, with a focus on air pollution and climate-related health risks (e.g. heatwaves and emissions);
 3. develop climate-resilient health systems that prioritize equity and social protection, climate-sensitive mental health risks, gender considerations and safeguarding risks in climate adaptation strategies with a focus on vulnerable and underserved communities.
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Enablers supporting the SFAs

Ensuring “leaving no one behind”

A society is only as healthy as its most at-risk members. When marginalized groups and vulnerable populations, including the refugees from Karabakh, can access health and social care services, this will result in fewer health disparities, inclusive economic participation, community inclusion and stronger social cohesion.

Addressing gender and equality

Gender-responsive health systems are fundamental to unlocking a nation’s full potential, with benefits that extend from household incomes to national development indicators. When women’s health needs are met, workforce participation grows and families prosper. When gender-based violence and violence against women and girls decline, communities become safer and more productive.

Enhancing digital health through digital transformation

Digital transformation rewires health systems to broader national development for efficiency: telemedicine erases geographic barriers, data analytics optimize limited resources, and innovation spillovers accelerate progress in access to health and care sectors.



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Implementing arrangements

The successful implementation of this CCS hinges on a cohesive, multi-level effort. The WHO Country Office in Armenia will lead the engagement at the country level, serving as the primary interface for the Government and partners. It will be responsible for day-to-day coordination, providing direct technical assistance and building context-specific and sustainable national capacity. The work in Armenia will be further supported by the technical units at the WHO Regional Office for Europe and, when necessary, by the normative and technical expertise of WHO headquarters, ensuring Armenia has access to global best practices and standards. To ensure impact and efficiency of the various investments in health, WHO will facilitate and convene national and international stakeholders advocating for and implementing health priorities in Armenia. In addition, WHO will initiate joint projects with other UN agencies and international and multilateral financial institutions to optimize resources and enhance impact, while also pursuing collaboration with partners within and beyond the health sector, including academia and civil society.

To ensure the continued relevance and effectiveness of WHO's support in Armenia, the implementation of the CCS will be guided by regular review, reporting and adaptive planning and learning processes. A high-level committee consisting of the senior leadership of the Ministry of Health, WHO and other relevant stakeholders will promote the highest level of political engagement and ensure that health remains at the top of national development agenda. This committee will monitor progress, align activities with national priorities and resolve bottlenecks, if any. An annual progress report outlining the achievements and identified challenges will be prepared during the span of the CCS. A final independent evaluation of the CCS will be conducted in its penultimate year (2029) to shape the next CCS.

The implementation of activities agreed within the framework of the CCS will be guided by WHO's commitments to gender equality and to the prevention of sexual misconduct, ensuring that safeguarding and survivor-centred standards are consistently applied across partnerships and coordination mechanisms. To ensure the sustainability of its support, WHO will actively support the Government towards diversified resource mobilization, creating clear opportunities for donor engagement to address critical funding gaps. Furthermore, WHO will improve stakeholder understanding of its mandate and scope, and increase the visibility of implemented activities through strategic, results orientated communications. Ultimately, all efforts will be channelled towards leaving a lasting legacy of strengthened national expertise and resilient health institutions in Armenia.

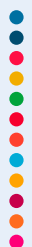
Monitoring and evaluation

A monitoring, evaluation and accountability framework of the CCS, consisting of action-based indicators, will be developed to permit quality improvement aligned with the agreed indicators for GPW 14 and EPW2. The CCS indicators will be adapted to reflect the national context, focus on national goals and priorities, and address issues most relevant to the country, while also fulfilling the national mandate to contribute to the global and regional monitoring effort. The CCS indicators will also complement existing regional and global broader health goals and the SDGs.



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Annex. Links between the strategic focus areas of the Country Cooperation Strategy (CCS), WHO global and regional priorities, and the United Nations Sustainable Development Cooperation Framework UNSDCF

Table A1 demonstrates the coherence and alignment among the WHO CCS for Armenia, 2026–2030, The WHO Fourteenth General Programme of Work, 2025–2028 (GPW 14), the Second European Programme of Work 2026–2030 (EPW2), and the Government of Armenia and the UNSDCF 2026–2030, highlighting the shared priorities and mutually reinforcing objectives that guide WHO’s work at the country level.

Table A1. Strategic linkages and alignment

Strategic focus area (SFA) of the CCS	GPW 14	EPW2	UNSDCF
SFA 1. Strengthening primary health care system to achieve universal health coverage	Strategic objective 3: Advance the primary health care approach and essential health system capacities for universal health coverage Strategic objective 4: Improve health service coverage and financial protection to address inequity and gender inequalities	Priority 5. Shaping the health systems of the future Area 1: Paradigm shifts in service delivery Area 2: Health and care workforce Area 3: Sustainable financing and affordable access Area 4: Access to medicines Area 5: Leveraging data and technology in the age of artificial intelligence	Outcome 1. By 2030, people, particularly the most vulnerable, enjoy higher-quality, inclusive and gender-responsive, and shock-responsive services and develop their full potential to lead healthy and resilient lives. Outcome 3. By 2030, climate change adaptation and mitigation, biodiversity protection, and sustainable natural resources management is strengthened, through gender-responsive, inclusive and innovative approaches. Outcome 4. By 2030, governance institutions and systems are more responsive, accountable and transparent, upholding the rule of law, human rights and gender equality.
SFA 2. Securing equitable access to health services through resilient primary health care systems	Strategic objective 2: Address health determinants and the root causes of ill health in key policies across sectors Strategic objective 4: Improve health service coverage and financial protection to address inequity and gender inequalities	Priority 1. Maximizing health security Area 3: Control infectious disease threats Priority 2. Tackling NCDs and shaping the drivers of health across sectors Area 1: Paradigm shifts in service delivery Area 2: Reducing preventable mortality from NCDs Area 3: Reducing treatable mortality and morbidity from NCDs	
SFA 3. Assuring demographic resilience through quality health across the life course	Strategic objective 4: Improve health service coverage and financial protection to address inequity and gender inequalities	Priority 3. Living and ageing in good physical and mental health Area 1: Supporting lifelong health for ageing well Area 2: Strengthening health and care systems for older persons	
SFA 4. Strengthening national health security around preparedness for, response to and recovery from emergencies	Strategic objective 5: Prevent, mitigate and prepare for risks to health from all hazards Strategic objective 6: Rapidly detect and sustain an effective response to all health emergencies	Priority 1. Maximizing health security Area 1: Anticipate and prepare for health emergencies Area 2: Detect and respond to health emergencies	
SFA 5. Prioritizing the environment and health nexus through a healthy climate ecosystem	Strategic objective 1: Respond to climate change, an escalating health threat in the 21st century	Priority 4. Driving climate–health action Area 1: Climate change adaptation Area 2: Climate change mitigation	

Notes: NCDs: noncommunicable diseases; SFA: Strategic focus area

The WHO Regional Office for Europe

The World Health Organization (WHO) is a specialized agency of the United Nations created in 1948 with the primary responsibility for international health matters and public health. The WHO Regional Office for Europe is one of six regional offices throughout the world, each with its own programme geared to the particular health conditions of the countries it serves.



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